INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nintedanib · continuation

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00372
Version 6

> Take the prescribed nintedanib dose twice daily every day. Reviews are organised in 28-day cycles; there is no planned tablet-free week. This phase starts only after at least 4 combination cycles and team approval.

Treatment stages

01 **Tablet-only continuation**
Tablet-only continuation

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	4
03	Symptoms and supportive care	8
04	Sources and further information	12

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Three medicines. Different jobs.

To continue controlling lung cancer after combination treatment, while benefit continues and side effects remain acceptable.

ORAL ANGIOKINASE INHIBITOR

Nintedanib

Nintedanib



Blocks VEGF, FGF and PDGF receptor signalling

Nintedanib blocks several signals involved in the growth of blood vessels that support a tumour. Its effects can also cause diarrhoea, liver changes, bleeding and wound-healing problems.

Blocks VEGF, FGF and PDGF receptor signalling → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Call early for diarrhoea or poor fluid intake; report bleeding, yellow eyes, dark urine, severe tummy pain or new neurological symptoms urgently.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

The trial found a progression-free benefit overall and an overall-survival benefit in the prespecified adenocarcinoma subgroup.

The licensed/NCCP population is adenocarcinoma after first-line chemotherapy. Overall-population PFS and adenocarcinoma-subgroup OS are labelled separately; this does not establish an independent benefit from maintenance-only initiation.

CHAPTER 02

Your schedule, tests and appointments

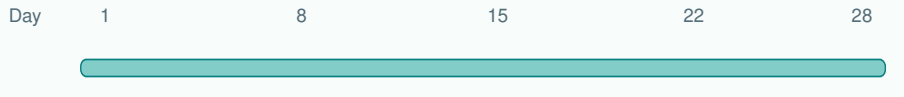
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment → review → next steps

Nintedanib · continuation

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TABLET-ONLY CONTINUATION

Tablet-only continuation

Ongoing

Twice daily every day;28-day review cycles while helping and tolerable. No fixed total number of cycles.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00372 v6 · official regimen · Vargatef current EMA product information

THE WHOLE TREATMENT JOURNEY

Nintedanib · continuation: treatment and checks

Adenocarcinoma non-small-cell lung cancer (NSCLC) · nintedanib-only continuation

Take the prescribed nintedanib dose twice daily every day. Reviews are organised in 28-day cycles; there is no planned tablet-free week. This phase starts only after at least 4 combination cycles and team approval.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00372 · SMPC	Confirm≥4 combination cycles and approval	Nintedanib twice daily	Continue every day	28-day review, no fixed total	Stop/change only on team advice

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Blood count / kidneys NCCP 00372 · SMPC	Recovery and baseline for this phase	Before each 28-day cycle	Extra tests if indicated	Review trend	Follow abnormalities
Liver / blood pressure NCCP 00372 · SMPC	Review liver tests and blood pressure (BP)	Liver at least monthly	BP as indicated; report symptoms	Additional monitoring for risk	Follow abnormalities
Scans / other care NCCP 00372 · SMPC	Individual disease plan	No fixed scan day	Report symptoms early	Assess benefit and tolerability	Agreed follow-up

What each check is for

1 **Confirm continuation**

After≥4 combination cycles

To continue controlling lung cancer after combination treatment, while benefit continues and side effects remain acceptable.

Ask / act: Confirm new prescription and monthly tests with the team.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.

STEP 2

Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.

STEP 3

Medicine review

Check all medicines and supplements with oncology/pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.

STEP 4

Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Nintedanib twice daily	 **Days 1–28** Continuous every day; no scheduled week off.
Safety and benefit review	 **Days 1–28** Keep the agreed monthly / cycle tests.

Symptoms can occur at any time: Side effects and infection do not follow a guaranteed day-by-day risk curve. Contact the team when symptoms occur.

Changes for safety: Treatment may be delayed, reduced or stopped. Do not make up missed doses or change your tablets yourself.

> **Why these bands stay broad:** Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Between-treatment notes

Day 1–28: Nintedanib every day

Take your prescribed capsule dose twice daily about 12 hours apart, including Day 1. There is no docetaxel in this phase. Use the current prescription and contact plan.

Support around the medicines

1 **Medicine review**

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nintedanib-only continuation · continuous twice daily

Active medicines: Nintedanib

Emetogenic risk: Minimal-to-low oral

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine oral antiemetic prophylaxis in minimal-to-low pathway; prescribe as-needed treatment after interaction/QT/constipation review.
- Give an early diarrhoea action plan with oral hydration and prescribed anti-diarrhoeal treatment; ongoing/severe symptoms need assessment and possible interruption.
- Continuous nintedanib includes Day 1 of every 28-day cycle. No docetaxel steroid premedication or automatic chemotherapy granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) course applies in this phase.
- Review P-gp interactions, liver tests, blood pressure (BP), bleeding, renal status, surgery/wound healing and contraception.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / neurological symptoms

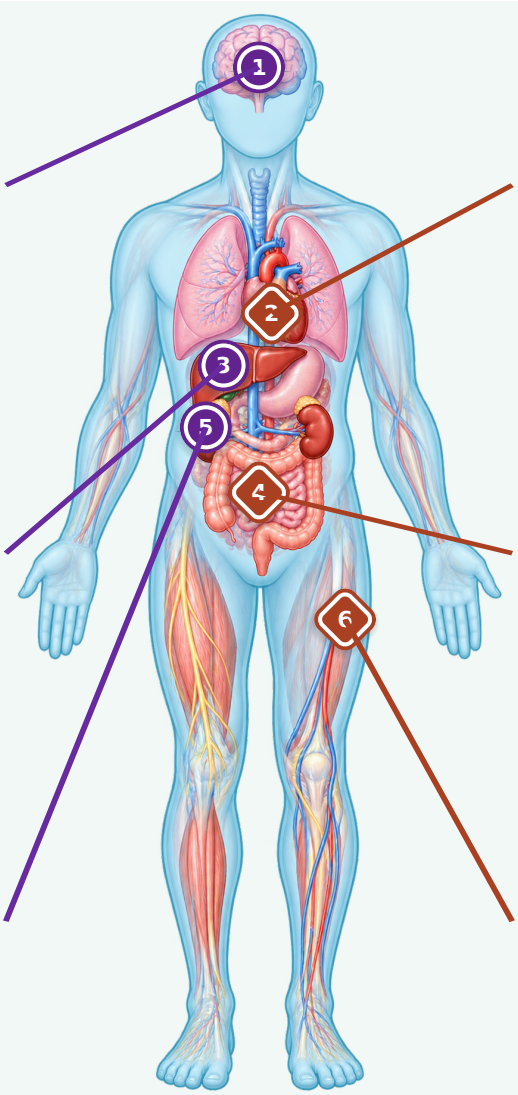
Nintedanib

New severe headache, confusion, seizure or visual change needs urgent advice.

2 Blood vessels / pressure

Nintedanib

Chest pain, sudden breathlessness or a swollen painful leg needs urgent assessment.



3 Liver

Nintedanib

Attend monthly liver tests; report yellow skin/eyes or dark urine.

4 Bowels / sickness

Nintedanib

Call early for diarrhoea or vomiting; severe abdominal pain is urgent.

5 Kidneys / urine

Nintedanib

Report swelling, foamy urine or much less urine.

6 Bleeding / wounds

Nintedanib

Report bleeding or poor wound healing. Tell oncology before procedures.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00372 v6 · official regimen**
   https://healthservice.hse.ie/documents/6913/372_Nintedanib_therapy.pdf
2. **Vargatef current EMA product information**
   https://www.ema.europa.eu/en/documents/product-information/vargatef-epar-product-information_en.pdf
3. **NCCP antiemetic classification v 6 and oral pathway**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCIS medical oncology antiemetic medicines**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **LUME-Lung 1 primary publication**
   https://pubmed.ncbi.nlm.nih.gov/24411639/